

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$37,626,974
Prenatal arm	\$11,855,074
Postnatal arm	\$25,771,900
Administrative load (8%)	\$3,010,158
Appropriation total (benefit + admin)	\$40,637,132
Sampling 90% CI	\$28,597,198 - \$46,656,749
Assumption band	\$22,576,382 - \$47,973,866

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$37,626,974	\$40,637,132	16,494
Universal · 6-mo postnatal	\$4,500	\$62,458,515	\$67,455,197	27,379
Universal · 12-mo postnatal	\$7,500	\$105,238,320	\$113,657,386	27,379

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$25,771,900
Total cost (12-month design)	\$63,398,874

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

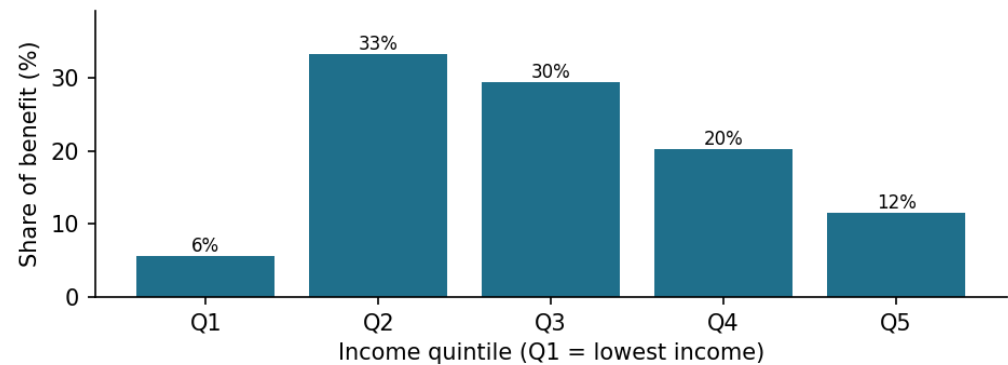
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$15,608,703
Prenatal / Postnatal	\$6,421,498 / \$9,187,205
Postnatal deferred to next FY	\$4,772,574
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$24,418,875 / \$15,608,703 / \$10,405,802

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	16,494
Expected pregnancies / infants	7,903 / 8,591
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,182	110,267	911	\$2,078,452	5.5%
Q2	\$57,026	110,263	5,499	\$12,543,940	33.3%
Q3	\$96,722	110,317	4,862	\$11,091,691	29.5%
Q4	\$154,809	110,231	3,329	\$7,593,745	20.2%
Q5	\$334,084	110,343	1,893	\$4,319,145	11.5%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$8,136,858	\$2,563,667	\$5,573,190	3,567
Honolulu	\$24,853,371	\$7,830,514	\$17,022,857	10,895
Kauai	\$2,686,303	\$846,369	\$1,839,934	1,178
Maui	\$1,950,443	\$614,523	\$1,335,920	855

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.